Paclitaxel Days 1,8 and 15 · relapsed or refractorySCLC

Treatment scope · NCCP 00621 NCCP 00621d: small-cell lung cancer that has returned or has not responded to previous treatment. This use is outside the Irish licence and requires the hospital off-label approval and consent process. This guide is for conventional paclitaxel alone.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Paclitaxel · Days 1, 8 and 15 of 28 · reviewed supportive care

Active medicines: Paclitaxel

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally	On treatment day Review QT, constipation and hepatic dosing; reconcile any steroid already prescribed
Metoclopramide	10 mg orally up to three times daily as needed	Subsequent days: duration to be confirmed Assess contraindications, renal function and interactions

Additional phase-specific support

- First paclitaxel dose: dexamethasone 8 mg IV, chlorphenamine 10 mg IV and famotidine 20 mg IV, each 30 minutes before treatment (NCCP suggested table).
- From Day 8 onward: chlorphenamine 10 mg IV and famotidine 20 mg IV 30 minutes before treatment. Routine dexamethasone may be omitted after the first dose; prior reaction or increased risk requires consultant review.
- Famotidine may be omitted under the consultant/local plan if appropriate. A prior reaction requires a separate rechallenge assessment; do not reuse the standard draft automatically.
- First-dose NCCP suggested premedication: dexamethasone 8 mg IV, chlorphenamine 10 mg IV and famotidine 20 mg IV 30 minutes before paclitaxel.
- From Day 8, dexamethasone may be omitted in the source suggested table; add or modify for increased risk or previous reaction under consultant guidance. Famotidine omission is also a consultant/local-policy decision requiring close observation.
- The conventional paclitaxel product information has a different generic premedication regimen. Reconcile the exact product, weekly NCCP pathway and local policy; never self-omit premedication.
- Low emetic risk; keep the antiemetic plan separate from reaction premedication. Consider analgesia for myalgia/arthralgia and early action on neuropathy.
- Confirm first-dose versus later-dose paclitaxel premedication. No Day 22 treatment in 00621.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Sources checked 2026-11-02 · SACTCheck v0.84.28 · Regimen-level drafts; prescriber and oncology-pharmacy verification required